

1. Administrative & Study Identification

Full Study Title: A Study of Rilvegostomig or Durvalumab Plus Chemotherapy for First-Line Treatment of Biliary Tract Cancer (ARTEMIDE-Biliary02) **Short Title/Acronym:** ARTEMIDE-Biliary02 (AB02) **Protocol Number:** D702NC00001 **NCT Number:** NCT07221253 **Sponsor Name:** AstraZeneca **Investigational Product:** Rilvegostomig (PD-1/TIGIT bispecific mAb) / Durvalumab **Phase of Development:** Phase III **Trial Status:** Not Yet Recruiting (Not yet recruiting participants) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the Overall Survival (OS) efficacy in the PDL1 ≥ 1% population of rilvegostomig in combination with gemcitabine plus cisplatin compared to durvalumab in combination with gemcitabine plus cisplatin as a first-line treatment for patients with advanced Biliary Tract Cancer (BTC). **Secondary Objectives:** To evaluate Overall Survival (OS) in the intent to treat (ITT) population, Progression-Free Survival (PFS) in the PDL1 ≥ 1% population, Progression-Free Survival (PFS) in the intent to treat (ITT) population, Objective Response Rate (ORR), and Duration of Response (DoR).

2.2 Background & Rationale To address the critical need for improved therapies in advanced Biliary Tract Cancer (BTC). This study investigates whether dual targeting of the PD-1 and TIGIT immune checkpoint pathways with the bispecific antibody rilvegostomig, added to standard chemotherapy, can provide a meaningful survival advantage over the current standard of care (durvalumab plus chemotherapy).

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Durvalumab + Chemotherapy) **Blinding:** Open-label **Randomization:** Randomized **Status:** Not Yet Recruiting

3.2 Planned Enrollment & Duration Target **NS:** 1,100 evaluable patients **Number of Sites:** Global, Multicenter **Estimated Study Start:** 04-Dec-2025 **Estimated Primary Completion:** 04-Jul-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically confirmed adenocarcinoma of the biliary tract, including intra-hepatic or extra-hepatic cholangiocarcinoma (CCA) and gallbladder carcinoma (GBC). 2. Unresectable locally advanced or metastatic BTC, previously untreated in the advanced disease setting. 3. Known PD-L1 status assessed at a central laboratory using an acceptable tumor sample. 4. Measurable disease by RECIST 1.1 criteria using CT or MRI and is suitable for accurate repeated measurements. 5. ECOG Performance Status of 0 or 1 with no deterioration (ie, ECOG PS > 1) over the previous 2 weeks prior to baseline at screening and prior to randomization. 6. Adequate bone marrow and organ function.

4.2 Exclusion Criteria 1. Ampullary carcinoma. 2. Any prior systemic therapy received for unresectable, locally advanced or metastatic BTC. 3. Any prior exposure to any other therapy targeting immune-regulatory receptors or mechanisms. 4. Any concurrent chemotherapy, radiotherapy, immunotherapy, investigational, biologic, or hormonal therapy for cancer treatment other than those under investigation in this study. 5. Active or prior documented autoimmune or inflammatory disorders requiring chronic treatment with steroids or other immunosuppressive treatment. 6. Active or ongoing interstitial lung disease/pneumonitis (of any grade), serious chronic gastrointestinal conditions associated with diarrhea, or active non-infectious skin disease (including any grade rash, urticaria, dermatitis, ulceration, or psoriasis) requiring systemic treatment.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Rilvegostomig or durvalumab administered in combination with gemcitabine and cisplatin (trade name: Platinol). **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing disease symptoms or chemotherapy-related side effects. **Prohibited:** Any concurrent chemotherapy, radiotherapy, immunotherapy, investigational, biologic, or hormonal therapy for cancer treatment other than those under investigation in this study.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Imaging (CT/MRI), PD-L1 Status Assessment

| | **Baseline** | Day 0 | Randomization, First Dose of Investigational Product | | **Interim** | Per Treatment Cycle | Safety Labs, Efficacy Assessment (RECIST 1.1 Tumor Imaging), AE Monitoring | | **End of Study** | Disease Progression | Final Vital Signs, Treatment Discontinuation, Survival Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Overall Survival (OS) in the PDL1 $\geq$ 1% population. **Measurement Method:** Time from randomization until the date of death due to any cause, evaluated centrally/locally per protocol.

7.2 Secondary & Exploratory Endpoints - Overall Survival (OS) in the intent to treat (ITT) population - Progression-Free Survival (PFS) in the PDL1 $\geq$ 1% population - Progression-Free Survival (PFS) in the intent to treat (ITT) population - Objective Response Rate (ORR) - Duration of Response (DoR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard clinical collection via patient evaluation, laboratory tests, and HCP interviews at each treatment cycle. **Serious Adverse Events (SAEs):** Safety signals reported strictly within standard trial timelines (typically 24 hours). **Data Monitoring Committee (DMC):** Independent safety monitoring mechanisms implemented by the sponsor to oversee trial safety.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy assessments; Safety Set for all adverse event evaluations. **Sample Size Justification:** Target sample of $N=1,100$ is powered to detect a statistically significant difference in Overall Survival (OS) between the rilvegostomig and durvalumab treatment arms. **Handling of Missing Data:** Right-censoring techniques (e.g., Kaplan-Meier methodology) will be used for time-to-event survival and progression endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Periodic onsite and remote monitoring visits to ensure integrity of RECIST 1.1 assessments and safety data. **Audit Trail:** Extensive electronic Case Report Form (eCRF) locking

procedures, query resolution, and data clarification forms via standard quality audit mechanisms.

11. Study Identifiers & Governance

Primary ID: D702NC00001 **Registry Number:** NCT07221253 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** ARTEMIDE-Biliary02
Official Regulatory Title: Phase III, Randomized, Open-label, Global, Multicenter Study of Rilvegostomig or Durvalumab in Combination With Chemotherapy as a First-line Treatment for Patients With Advanced Biliary Tract Cancer

12. Clinical Framework (Oncology/BTC Specific)

Primary Condition: Biliary Tract Cancer (BTC) **Diagnosis Threshold:** Histologically confirmed unresectable locally advanced or metastatic BTC **Medical Methodology:** First-Line Immunotherapy (PD-1/TIGIT vs PD-L1) + Chemotherapy **Optimization Target:** Overall Survival (OS) and Tumor Response

13. Study Procedures & Methodology

Management Pathway: Standard oncology pathway for advanced BTC management **Education Protocol (HCP):** Protocol-specific training, safety reporting, and RECIST 1.1 evaluation criteria **Education Protocol (Patient):** Onsite training regarding study drug administration, side effect management, and trial schedule **Quality Audit Mechanism:** Source data verification, clinical site monitoring, and central laboratory assessments for PD-L1

14. Observation & Follow-Up Schedule

Total Duration: Estimated 43 months (Study completion expected July 2029) **Onsite Visit Cadence:** Every 3-4 weeks aligning with regular chemotherapy/immunotherapy infusion cycles **Remote Monitoring Frequency:** Periodic survival follow-up post-disease progression **Checklist Review Interval:** Regular tumor imaging assessments (e.g., every 8-12 weeks) according to RECIST 1.1

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				